



STROKE UNIT PATIENT SATISFACTION MONITORING PLAN

Framework for Structured, Accessible, and Continuous Monitoring of Patient and Caregiver
Experience on the Stroke Unit

Applicable Units: Acute Stroke Unit • Neuro-ICU • Stroke Prevention / Outpatient Follow-Up Clinic

Document Type: Stroke Program Quality & Patient Experience Plan (Companion to Stroke Unit Delirium Screening Protocol and MDR Documentation Form)

Guideline Basis: CMS HCAHPS / Hospital Value-Based Purchasing; AHA/ASA Get With The Guidelines–Stroke; The Joint Commission / DNV Stroke Center Certification Standards

Prepared for review by: Stroke Program Medical Director • Stroke Coordinator • Patient Experience Committee • Quality & Patient Safety Committee

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1. Purpose and Scope

1.1 Purpose

This plan establishes a structured, continuous approach to monitoring patient and caregiver satisfaction and experience on the stroke unit. It addresses a gap common to standard hospital-wide satisfaction programs: stroke patients are disproportionately excluded from or underrepresented in standard surveys because of aphasia, cognitive impairment, neglect, or fatigue — the very deficits the stroke unit exists to treat.

1.2 Scope

Applies to all patients admitted to the Acute Stroke Unit or Neuro-ICU under the stroke service, and to the Stroke Prevention/Outpatient Follow-Up Clinic for post-discharge experience capture. Covers hospital-mandated survey participation, stroke-specific supplemental measures, real-time in-unit feedback, and complaint/grievance monitoring.

1.3 Relationship to Other Stroke Unit Documents

This plan complements the Stroke Unit Delirium Screening Protocol and Multidisciplinary Rounds (MDR) Documentation Form. Discharge-readiness and communication-experience findings surfaced through this plan should be discussed at multidisciplinary rounds and fed back into unit-level practice changes.

2. Guiding Framework

This plan uses the Donabedian structure-process-outcome model as its organizing framework for patient experience, treating experience as a distinct quality dimension alongside clinical outcomes rather than a subordinate or “soft” metric:

Dimension	Application to Stroke Unit Experience
Structure	Availability of communication aids, private family conference space, aphasia-friendly materials, adequate staffing for bedside rounding.
Process	How care is delivered and communicated: shared decision-making, discharge preparation, responsiveness to call bell, communication access for aphasic patients.
Outcome	Patient- and caregiver-reported satisfaction, discharge readiness scores, complaint rates, and, where feasible, association with readmission and functional outcome measures.

Why This Matters Clinically, Not Just Administratively

Published stroke outcomes research has examined the relationship between patient satisfaction scores and clinical outcomes; regardless of the strength of that association, patient experience is tracked here as a legitimate quality endpoint in its own right, particularly because communication access failures for aphasic patients are independently linked to safety and care-quality concerns.

3. Accreditation and Regulatory Basis

Framework	Relevance
CMS HCAHPS / Hospital Value-Based Purchasing	Mandatory, standardized inpatient experience survey; ties hospital-level results to CMS reimbursement. Applies to all inpatients, including stroke, but is not stroke-specific and does not by itself capture communication-access adequacy.
AHA/ASA Get With The Guidelines–Stroke (GWTG-Stroke)	National stroke quality registry; institutions may add locally defined patient experience measures as supplemental quality indicators.
The Joint Commission / DNV Primary, Thrombectomy-Capable, and Comprehensive Stroke Center Certification	Certification standards expect patient- and family-centered care processes; this plan provides documented evidence of a structured experience-monitoring program for survey purposes.
CMS Conditions of Participation (42 CFR §482)	General patient rights and communication requirements, including provision of effective communication for patients with communication impairments.

Certification Note

Route this document through your Stroke Coordinator for inclusion in your certification evidence binder under the applicable patient- and family-centered care standard, and confirm your organization's specific HCAHPS vendor/reporting workflow before distribution.

4. Why Standard Monitoring Under-Counts Stroke Patients

Generic hospital satisfaction programs assume the patient can independently read, comprehend, and complete a written or telephone survey. This assumption fails routinely in stroke populations:

- Aphasia (expressive, receptive, or global) can prevent patients from reading survey items or articulating responses, regardless of their actual satisfaction with care.
- Hemineglect can cause patients to miss or misread portions of a written survey.
- Cognitive impairment or post-stroke fatigue reduces the reliability of in-hospital survey responses, particularly early in the admission.
- Patients who cannot complete a survey are frequently simply excluded from the sample rather than flagged for proxy response — this silently skews results toward the least-impaired patients and can mask communication-access problems that most need attention.

Core Principle

No stroke patient should be excluded from experience monitoring solely because of a communication or cognitive deficit. Where the patient cannot respond independently, a documented proxy pathway (Section 5.3) is used instead of exclusion, and the proxy status is recorded alongside the response so results can be interpreted correctly.

5. Monitoring Instruments and Layers

Experience is monitored through five complementary layers rather than a single survey, so that no single instrument's blind spots go uncorrected.

Layer	Instrument	Purpose
1. Hospital-wide mandatory survey	HCAHPS (post-discharge)	Regulatory baseline; comparability across hospital and national benchmarks.
2. Stroke-specific supplemental survey	Locally adapted stroke PREM (discharge-readiness, shared decision-making, communication access, caregiver preparedness)	Captures stroke-specific experience domains HCAHPS does not address.
3. Real-time in-unit feedback	Structured bedside leadership rounding; communication boards for aphasic patients	Immediate service recovery; catches problems while still correctable during the stay.
4. Post-discharge follow-up	Stroke Prevention Clinic / nurse navigator phone call (typically ~30 days)	Captures considered feedback once the patient is medically stable enough to reflect meaningfully.
5. Passive signal tracking	Complaint/grievance log, patient relations referrals	Surfaces acute dissatisfaction that would not appear in scheduled surveys.

5.1 Hospital-Wide Survey (HCAHPS)

Administered per the hospital's standard HCAHPS vendor process; no stroke-specific modification is made to this instrument, as doing so would compromise regulatory comparability. Stroke unit results are extracted from the hospital-wide dataset and reviewed separately by the Stroke Quality Committee (Section 8).

5.2 Stroke-Specific Supplemental Survey

A short, locally developed supplemental survey (typically 8–12 items) is administered at discharge, covering domains not well captured by HCAHPS: clarity of stroke diagnosis and prognosis explanation, involvement in care and discharge decisions, caregiver/family preparedness for the transition home, and adequacy of communication support for patients with aphasia.

5.3 Proxy Response Pathway

1. At the time any survey is offered, the bedside RN or care coordinator screens for the patient's ability to respond independently (using the communication status already documented on the Stroke Unit Delirium Screening Form reference exam).
2. If the patient can respond independently, even with communication support (e.g., communication board, extra time), the patient responds directly.

3. If the patient cannot respond independently, the primary family member or caregiver is invited to respond as a documented proxy, and the response is explicitly flagged as proxy-completed in the tracking system, not merged indistinguishably with patient-completed responses.
4. If no caregiver is available and the patient cannot respond, the case is logged as non-respondent (communication barrier) rather than silently dropped, so the true proxy/non-response rate is visible on the unit dashboard (Section 11).

6. Data Collection Workflow and Timing

Layer	Timing	Administered By	Response Pathway
HCAHPS	Post-discharge (per hospital vendor timeline)	Hospital survey vendor	Patient or documented proxy (per hospital-wide policy)
Stroke supplemental PREM	At or just before discharge	Bedside RN / Stroke Coordinator	Patient direct, communication-supported, or proxy per Section 5.3
Bedside rounding	Weekly minimum; daily on high-census or high-acuity days	Charge nurse / unit leadership	Direct conversation; documented in rounding log
Communication board feedback	Ongoing throughout stay	Bedside RN / SLP	Direct patient input via picture/symbol board
Post-discharge follow-up call	~30 days post-discharge	Stroke nurse navigator / Stroke Prevention Clinic	Patient or caregiver by phone
Complaint/grievance log	Continuous	Patient Relations / unit staff	Any patient, family, or staff-initiated report

7. Aphasia-Friendly and Accessible Survey Design

The stroke-specific supplemental survey (Section 5.2) and bedside feedback tools are designed using accessible-communication principles so more patients can respond directly rather than defaulting to proxy:

- Short, single-idea sentences; avoid compound or negatively phrased questions
- Large, high-contrast font; generous white space; one question per line
- Visual analog or simple face/symbol rating scales as an alternative to text-based Likert scales
- Yes/No or pointing-based response formats available as an alternative to open-ended questions
- Extra, unhurried time for response; never administered during a busy clinical moment
- Speech-language pathology consultation on survey format for patients with significant aphasia, when available

- Materials available in the patient's preferred language, with interpreter support for administration when needed

8. Governance and Reporting

Results from all five monitoring layers are consolidated and reviewed by a standing committee rather than left in separate departmental silos:

- Stroke Quality Committee (or Patient Experience Committee with stroke representation) reviews consolidated results monthly
- HCAHPS stroke-unit-level results, supplemental PREM domain scores, proxy-response rate, and complaint volume are reviewed together, not in isolation
- Findings are reported to the Medical Executive Committee and incorporated into stroke center certification quality reporting on the certification body's required cadence
- Negative or below-threshold findings trigger a documented improvement action per Section 10

9. Roles and Responsibilities

Role	Responsibility
Bedside RN	Screens communication status before offering any survey; administers stroke supplemental survey or facilitates proxy response; documents communication board interactions.
Charge Nurse / Unit Leadership	Conducts structured bedside rounding; logs real-time feedback and immediate service recovery actions.
Speech-Language Pathology	Advises on communication access for aphasic patients; supports aphasia-friendly survey administration when needed.
Stroke Nurse Navigator / Stroke Prevention Clinic	Conducts post-discharge follow-up calls; captures considered feedback once the patient has stabilized.
Stroke Coordinator	Maintains this plan; consolidates results across all five layers; prepares reporting for the Stroke Quality Committee.
Patient Relations	Manages formal complaint/grievance intake and tracking; feeds volume and themes into the consolidated report.
Stroke Quality / Patient Experience Committee	Reviews consolidated results monthly; approves improvement actions; reports to Medical Executive Committee.

10. Quality Improvement Loop

Findings are acted on using a standard Plan-Do-Study-Act (PDSA) cycle rather than reviewed passively:

5. Plan: Identify the lowest-scoring domain or theme from the consolidated monthly report (e.g., discharge preparedness, communication access, responsiveness).
6. Do: Implement a targeted, small-scale intervention (e.g., a structured discharge summary tool, revised communication board content, adjusted rounding frequency).
7. Study: Re-measure the relevant domain over a defined period (typically 4–8 weeks) using the same instrument layer that identified the issue.
8. Act: Adopt, adjust, or abandon the intervention based on the re-measurement; document the outcome and carry sustained changes into standard unit practice and, where relevant, into the MDR Documentation Form or unit orientation materials.

11. Metrics and Targets

Metric	Data Source	Target
HCAHPS stroke-unit-level top-box score (communication domains)	HCAHPS	Meet or exceed hospital-wide benchmark
Stroke supplemental PREM response rate (direct + proxy combined)	Supplemental PREM	≥ 80% of discharges
Proxy-response rate (tracked, not penalized — monitored for pattern)	Supplemental PREM	Tracked monthly; reviewed for communication-access gaps
Non-response due to communication barrier, with no available proxy	Supplemental PREM	< 5% of eligible discharges
Discharge-readiness domain score	Supplemental PREM	≥ 80% top-box
Complaint/grievance rate related to communication or discharge process	Patient Relations log	Downward trend quarter over quarter
Time from bedside-rounding-identified issue to documented service recovery action	Rounding log	Same shift where feasible

12. Documentation and Data Management

- Survey responses, proxy status, and rounding log entries are stored in the hospital's designated patient experience platform or EHR module, not in free-standing spreadsheets, to preserve auditability.

- Proxy responses are tagged as such in the data record and reported separately from direct-patient responses in trend analysis, while still being included in overall response-rate calculations.
- Complaint/grievance records follow the hospital's standard Patient Relations documentation and confidentiality requirements.
- Aggregate, de-identified results only are presented at the Stroke Quality Committee and in certification reporting; individual patient responses are not shared outside the direct care and quality review process.

13. Review, Revision, and Approval

This plan is reviewed annually, or sooner if triggered by a change in HCAHPS methodology, stroke center certification requirements, or a sustained shift in monitored metrics. Revisions require review and approval by the Stroke Program Medical Director, Stroke Coordinator, and Patient Experience / Quality & Patient Safety Committee.

Version	Date	Summary of Change	Approved By
1.0	[Insert Date]	Initial release	[Insert Name/Title]

14. References

1. Centers for Medicare & Medicaid Services. HCAHPS: Patients' Perspectives of Care Survey / Hospital Value-Based Purchasing Program.
2. Donabedian A. The Quality of Care: How Can It Be Assessed? JAMA.
3. Sorinmade OA, et al. Patient-Reported Experience Measures in Stroke Care: A Systematic Review. Stroke (AHA/ASA journal).
4. Rajendran V, et al. Improving Patient Experience and Discharge Readiness in Acute Stroke Care Through a Patient-Oriented Discharge Summary.
5. Stroke Association / NHS England. Patient Reported Experience Measures (PREMs) Survey — including aphasia-friendly reporting formats.
6. American Heart Association/American Stroke Association. Get With The Guidelines—Stroke Program Overview.
7. Is Higher Patient Satisfaction Associated With Better Stroke Outcomes? American Journal of Managed Care.

Before Finalizing

Insert your organization's HCAHPS vendor details, EHR/patient-experience platform name, stroke supplemental survey item set, and committee approval signatures before distribution. Confirm proxy-response documentation workflow with your Patient Experience/Patient Relations department, and pilot the aphasia-friendly supplemental survey with Speech-Language Pathology before full rollout.